

Overall impression

This is a well-styled two-sided product (doctor dashboard + patient portal) with a clean teal/off-white clinical aesthetic, card-based layout, and genuinely good density of oncology-specific data (CA 15-3, CEA, Gleason scores, PI-RADS, PSA velocity). That's rare — most demo healthcare UIs stop at generic vitals. The "OnCoTrack Clinician Brief" concept (diagnosis → treatment intent → milestone → evidence reviewed) is a strong differentiator. But there are real issues in metric design, information redundancy, safety-critical UI patterns, and consistency between the doctor and patient views.

1. Doctor Dashboard

Strengths: clear hierarchy, welcoming header, obvious search, patient cards are scannable.

Problems:

- **"Daily Consultations: 0" reads as broken, not empty.** A zero-state metric on a primary KPI card looks like a bug on first glance, especially sitting next to "Total Patients: 4." Either hide zero-value KPIs behind a friendlier empty state ("No consultations yet today — start one?") or make the card actionable.
 - **Mismatched time windows across the three cards** (Today / All Time / Today) is confusing at a glance — a doctor scanning quickly may misread "4" as today's patient count. Standardize the scope or label it more visually (e.g., consistent pill color per scope).
 - **Delete icon on every patient card, one tap away from the open/view icon.** This is a serious safety issue: two small circular icons of similar size sitting side-by-side, where one is destructive (delete a patient record) and one is navigational. A misclick risk here is high, and there's no visible undo/confirm shown. This needs a confirmation modal at minimum, and ideally delete shouldn't live on the main list at all — bury it in a detail-view menu.
 - **"Hours Logged: 3.7h"** — unclear what this measures (logged-in time? consultation time?) and why a doctor needs this front-and-center above their actual patients. Feels like a generic SaaS productivity metric bolted onto a clinical tool.
 - No filtering/sorting on the patient list (by risk, next appointment, flagged markers) — for an oncology panel, "who needs attention today" is more useful than a flat list.
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2. Patient Detail View — Doctor Side

Strengths: The clinician brief is genuinely thoughtful — diagnosis date, treatment intent, next milestone as color-coded pills is a nice pattern. "Evidence Reviewed" as checked bullet items gives a clean audit trail feel.

Problems:

- **Redundant vitals sections.** "Staff-Recorded Vitals" and "Vitals Snapshot" both show overlapping data (BP appears in both; temp/pulse/spo2 in one, HbA1c/glucose/cholesterol in the other) with no clear visual link explaining why they're separate. A clinician will wonder if these are two different data sources, two different time points, or just a UI duplication bug. This needs either merging or an explicit label like "Latest labs (external)" vs "In-clinic vitals."
 - **Oncology markers lack trend/reference context.** "CA 15-3: 17 U/mL — NORMAL" is good, but for an oncology surveillance tool, a single most-recent value with a static "NORMAL" tag undersells what matters most clinically: the trend over time. A sparkline or delta ("↓ from 22 three months ago") would be far more valuable than a badge, especially for markers like PSA, CEA, CA 15-3 that oncologists track velocity on, not just single readings.
 - **"Free PSA: 11% — LOW"** sitting next to three "NORMAL" tags is the single most clinically salient thing on Raman Iyer's page, but it's visually equal-weighted with everything else. Abnormal values should be visually escalated (color, position, or a flag), not just color-coded text buried in a 2x2 grid.
 - **Private Clinical Notes are just... visible.** No blur/expand-to-view friction, no indication of who else can see this. If this is meant to be truly private (vs. the patient-facing Doctor Notes seen in Image 4), the UI should differentiate access levels more strongly — right now a private note and a patient-facing note look nearly identical in card style.
 - **"Check-In History — No pre-visit forms yet"** takes up a full-width section for an empty state. Consider collapsing empty sections by default or moving them lower.
 - **Layout inconsistency:** Maya Srinivasan's page and Raman Iyer's page have different card widths/positioning for functionally identical data (e.g., clinician summary and markers swap proportions slightly) — worth auditing the grid system for consistency across patients.
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3. Patient-Facing Health Dashboard

This is where the split between doctor and patient experience raises the most questions.

Strengths: the digital "Health Card" with tap-to-flip and fingerprint watermark is a nice trust/security visual metaphor. Doctor Notes list with expand chevrons is clean.

Problems — and this is the biggest one:

- **A Stage IV metastatic cancer patient's top-of-page metrics are Blood Glucose, Blood Pressure, HbA1c, and Cholesterol — all green, all "normal," all "improved."** For a patient with metastatic sigmoid colon adenocarcinoma on active systemic chemotherapy (FOLFOX + bevacizumab), these are not the metrics that matter most, and leading with four cheerful green "improved" badges risks giving a false sense of reassurance about a serious, active cancer diagnosis. This is a genuine patient-safety/communication concern, not just a design nitpick — the oncology care plan (the actually critical information) is visually secondary, below the fold of the "everything is fine" metrics.
- **Emotional tone mismatch.** "improved ↑" styling and bright green normal badges next to "Stage IV" and "Metastatic" reads dissonant. Patient-facing oncology tools need much more careful visual tone-setting than a general wellness app — this currently feels like a fitness-tracker template reused for cancer care.
- Same duplication issue as the doctor side: is "Blood Glucose 96 normal" from the same source as the doctor's "Vitals Snapshot"? If so, showing raw lab numbers to a patient without context/interpretation from their oncologist could cause anxiety or confusion (e.g., "why is my cholesterol being tracked when I have colon cancer?" is a fair question a patient might ask).
- CEA is mentioned in the care plan text ("tracking CEA, blood counts, and liver imaging") but isn't shown anywhere as an actual number/trend on this page — the one marker that matters most for this patient's colorectal cancer monitoring is described but not visualized, while cholesterol gets its own KPI card.

4. Cross-cutting concerns

Security/privacy (important for a real product):

- Full patient names, IDs, and diagnoses are rendered in plaintext with no apparent screen-privacy mode, and the doctor's ID (DR-ONC-RAO) and patient IDs (NRV-

ONC-00x) are sequential/guessable — worth confirming these aren't used as the sole auth/lookup key anywhere.

- No visible audit trail UI for who viewed/edited a patient record, which matters a lot for oncology data and any HIPAA/DPDP-style compliance story.
- The delete button on the dashboard patient cards (Image 1) with no confirmation step is a compliance and safety liability as much as a UX one — deleting an oncology patient record should be a heavily gated action (reason, confirmation, possibly admin approval), not a one-tap icon.

Consistency:

- Font/spacing/badge styles are consistent (good design system discipline), but the *information architecture* between doctor and patient views doesn't obviously map — a patient can't easily see what their doctor sees (e.g., the oncology markers, evidence reviewed), and the doctor's view doesn't show what the patient dashboard emphasizes (glucose/cholesterol trends). If these are meant to be two windows into the same record, they should visually rhyme more.

Accessibility: color-only status coding (green "normal," red "low") without icon or text redundancy could be a problem for colorblind users — worth checking contrast ratios on the light green/mint backgrounds too.

Priority fixes if I had to pick three

1. **Rework the patient-facing dashboard's top metrics** so the oncology care plan and cancer-relevant markers (CEA, staging, next scan) lead, not generic vitals — this is a clinical-communication risk, not just aesthetics.
2. **Add confirmation + permission gating to patient deletion** on the doctor dashboard.
3. **Resolve the vitals duplication** (Staff-Recorded Vitals vs. Vitals Snapshot) with clear source/timestamp labeling so clinicians aren't left guessing which number is current.

Here's the sample version. Key changes from the original:

1. **Oncology care plan is now the hero** — diagnosis, stage, care phase, treatment, and next step lead the page, in a dark teal band that visually signals "this is the important thing," instead of sitting below four green wellness cards.

2. **CEA gets a trend chart, not a badge.** This is the marker that actually matters for this patient's monitoring — a single "normal/improved" tag hides the fact that it's been dropping since diagnosis, which is the story a patient (and their family) would actually want to see.
3. **Glucose/BP/HbA1c/cholesterol moved to a collapsed "General health metrics" section,** closed by default. They're still available — just not competing with cancer-specific data for attention, and no more "↑ improved" cheerfulness sitting next to "Stage IV."
4. **Tone shift in copy:** plain, second-person, calm language ("You're on modified FOLFOX plus bevacizumab...") instead of dashboard-speak, which matters more for a Stage IV patient than for a fitness app.

The same principle — lead with what's clinically significant, de-emphasize what isn't, show trend not just status — would apply just as well to the doctor-side pages if you want me to mock those up next (e.g., flagging Raman Iyer's low Free PSA instead of it sitting equal-weight next to three "normal" tags).

Here is a sample

